

1. Administrative & Study Identification

Full Study Title: An Open-Label Phase 1B/II Study of Glofitamab and Atezolizumab or Polatuzumab Vedotin in Adult Patients With Relapsed/Refractory B-Cell Non-Hodgkin's Lymphoma **Official Title:** An Open-Label, Multi-Center, Phase IB/II Study of Glofitamab and Atezolizumab or Polatuzumab Vedotin (Plus a Single Pre-Treatment Dose of Obinutuzumab) in Adult Patients With Relapsed/Refractory B-Cell Non-Hodgkin's Lymphoma **Short Title/Acronym:** Glofit-Pola in R/R LBCL (Protocol NP39488) **Protocol Number:** NP39488 **NCT Number:** NCT03533283 **Sponsor/Company Name:** Hoffmann-La Roche (Roche) **Trial Status:** Active, Not Recruiting **Investigational Product:** Glofitamab (Columvi) in combination with Atezolizumab or Polatuzumab Vedotin (Polivy); Obinutuzumab (Gp) **Phase of Development:** Phase I/II (PHASE1, PHASE2) **Disease Area:** Non-Hodgkins Lymphoma **Medical Monitor:** Hoffmann-La Roche Clinical Operations / Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess Dose Limiting Toxicities (DLTs), the maximum tolerated dose (MTD), and/or recommended Phase II dose (RP2D) for the combination treatments, and to evaluate the best objective response rate (ORR) as assessed by an Independent Review Committee (IRC). **Secondary Objectives:** To evaluate general safety, tolerability, pharmacokinetics (PK), pharmacodynamics, progression-free survival (PFS), overall survival (OS), best ORR as measured by Investigator, best Complete Response (CR) rate via FDG-PET/CT, duration of response (DOR), and duration of complete response (DOCR). **Sub-study Objective:** The study includes an additional open-label imaging feasibility sub-study using a tracer in adult participants with relapsed/refractory B-cell non-Hodgkin's lymphoma to image CD8+ T-cells at baseline and after treatment with glofitamab.

2.2 Background & Rationale To address the significant unmet medical need for more effective therapies in patients with relapsed/refractory (R/R) large B-cell lymphoma (LBCL), including high-grade B-cell lymphoma (HGBCL) and in patients who have failed prior therapies. Glofitamab (a CD20xCD3 bispecific antibody) and polatuzumab vedotin (a CD79b-directed antibody-drug conjugate) offer complementary modes of action aiming to improve durable complete remission rates.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose-finding and expansion) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Targeted Enrollment: 211 participants **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 08-May-2018 **Estimated Primary Completion:** 16-Oct-2026

4. Subject Selection (Eligibility Criteria)

4.1 Main Inclusion Criteria 1. Adult patients (Age ≥ 18 years) with histologically-confirmed hematologic malignancy expected to express CD20 (Relapsed/Refractory B-Cell Non-Hodgkin's Lymphoma). 2. Must have relapsed after or been refractory to ≥ 1 prior treatment regimen; no available treatment options expected to prolong survival or refused chemo/SCT. 3. Target populations include: Grades 1-3b R/R follicular lymphoma (FL), marginal zone lymphoma (MZL), diffuse large B-cell lymphoma (DLBCL) including NOS and transformed FL, primary mediastinal large B-cell lymphoma (PMBCL), and HGBCL (double-hit, triple-hit, and NOS). 4. At least one measurable target lesion. 5. Fresh pre-treatment biopsy (or an archived biopsy from a metastatic lesion ≤ 6 months old not confounded by major events). 6. Eastern Cooperative Oncology Group (ECOG) performance status of 0-2. 7. Adequate organ function (liver, hematological, renal). 8. Negative test results for hepatitis B virus (HBV), hepatitis C virus (HCV), and human immunodeficiency virus (HIV). *Imaging Substudy Specific:* At least two measurable target lesions; able to provide two fresh tumor biopsies (baseline and on-treatment).

4.2 Main Exclusion Criteria 1. Chronic Lymphocytic Leukemia (CLL), acute lymphoblastic leukemia (ALL), lymphoblastic lymphoma, Richter's transformation, CD20-positive ALL, Burkitt lymphoma, or lymphoplasmacytic lymphoma. 2. Current $\geq$ Grade 1 peripheral neuropathy (only for participants in the polatuzumab vedotin arm). 3. Known active infection, or reactivation of a latent infection within 4 weeks prior to Obinutuzumab infusion. 4. History of confirmed progressive multifocal leukoencephalopathy (PML), leptomeningeal disease, or current/past history of central nervous system (CNS) lymphoma or CNS disease. 5. Major surgery or significant traumatic injury ≤ 28 days prior to infusion. 6. Significant cardiovascular or pulmonary disease; active or history of autoimmune disease or immune deficiency; history of idiopathic pulmonary fibrosis, organizing pneumonia, or pneumonitis. 7. Prior allogeneic stem cell transplant (SCT), or autologous SCT within 100 days prior to infusion. 8. Documented refractoriness to an obinutuzumab-monotherapy regimen. 9. Ongoing corticosteroid use >25 mg/day

of prednisone equivalent within 4 weeks prior to/during study treatment, or systemic immunosuppressive medication. 10. Administration of a live, attenuated vaccine within 4 weeks prior to infusion. *Imaging Substudy Specific:* Circulating lymphoma cells; prior splenectomy or functional asplenia.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Obinutuzumab:** 1,000 mg given on Cycle 1 Day 1 to mitigate Cytokine Release Syndrome (CRS) risk. * **Polatuzumab Vedotin:** 1.8 mg/kg on Cycle 1 Day 2, and Day 1 of Cycles 2–6. * **Glofitamab:** Step-up dosing in Cycle 1 (Day 8: 2.5 mg; Day 15: 10 mg), followed by the target dose of 30 mg on Day 1 of Cycles 2–12. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** 21-day cycles. Polatuzumab vedotin given for up to 6 fixed-duration cycles; Glofitamab given for up to 12 cycles.

5.2 Concomitant & Prohibited Therapy Permitted: Corticosteroids as premedication prophylaxis for Cytokine Release Syndrome (CRS) and standard supportive care. **Prohibited:** Concurrent administration of other investigational agents, standard anti-cancer radiotherapy/chemotherapy, or live vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline PET/CT Imaging, Biopsies
Baseline	Cycle 1 (Days 1-21)	First dose of Obinutuzumab (Day 1), Polatuzumab vedotin (Day 2), and Glofitamab step-up dosing (Days 8, 15); strict CRS monitoring
Interim	Cycles 2-12	Q3W IV Infusions; Safety Labs, Efficacy Assessment (FDG-PET/CT scans), PK/PD monitoring
End of Study	Follow-up	End of treatment visit, survival follow-up, disease progression monitoring

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. Dose Limiting Toxicities (DLTs) to determine Maximum Tolerated Dose (MTD) / Recommended Phase II Dose (RP2D). 2. Best Objective Response Rate (ORR) as Measured by Independent Review Committee (IRC).

7.2 Secondary & Exploratory Outcomes 1. Best ORR as Measured by Investigator. 2. Best Complete Response (CR) Rate, as Assessed by Fluorodeoxyglucose-Positron Emission Tomography/Computed Tomography (FDG-PET/CT) Scan. 3. Duration of Complete Response (DOCR). 4. Progression-Free Survival (PFS) and Overall Survival (OS). 5. Duration of Response (DOR).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring with a specific focus on Dose Limiting Toxicities (DLTs), Cytokine Release Syndrome (CRS), infections, and cytopenias. CRS events are mitigated by pre-treatment with obinutuzumab. **Serious Adverse Events (SAEs):** Captured throughout the treatment and follow-up periods. **Data Monitoring Committee (DMC):** Internal and independent review boards overseeing patient safety and dose-escalation limits.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety-evaluable population (patients receiving ≥ 1 dose of study drug); Efficacy-evaluable population. **Sample Size Justification:** Scaled appropriately to determine DLTs/MTD and ensure sufficient power to observe meaningful ORR/CR rates across histologic subgroups. **Handling of Missing Data:** Time-to-event endpoints calculated using Kaplan-Meier methodology, with patients without events censored at the date of their last evaluable assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: Conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring of clinical data, strict CRS grading adherence, and drug accountability. **Audit Trail:** Electronic Case Report Form (eCRF) locking and independent IRC radiological review for unbiased tumor assessments.

11. Study Identifiers & Governance

Primary ID: NP39488 **Registry Number:** NCT03533283 **Sponsor/Lead Organization:** Hoffmann-La Roche (Roche) **Study Title:** Glofit-Pola in R/R LBCL **Official Regulatory Title:** An Open-Label, Multi-Center, Phase IB/II Study of Glofitamab and Atezolizumab or Polatuzumab Vedotin (Plus a Single Pre-Treatment Dose of Obinutuzumab) in Adult Patients With Relapsed/Refractory B-Cell Non-Hodgkin's Lymphoma

12. Clinical Framework (Lymphoma Specific)

Primary Condition: Relapsed/Refractory B-Cell Non-Hodgkin's Lymphoma **Diagnosis Threshold:** Refractory to or relapsed after ≥ 1 prior line of systemic therapy. **Medical Methodology:** T-cell engaging bispecific antibody + antibody-drug conjugate (ADC)

targeted therapy. **Optimization Target:** Complete Response (CR) without prohibitive Grade 3+ CRS or DLTs.

13. Study Procedures & Methodology

Management Pathway: Obinutuzumab pretreatment followed by step-up dose escalation. **Education Protocol (HCP):** Strict adherence to Cytokine Release Syndrome (CRS) mitigation, grading, and prompt administration of tocilizumab and/or corticosteroids if required. **Education Protocol (Patient):** Awareness of systemic inflammatory symptoms (fever, hypotension) typically occurring in Cycle 1. **Quality Audit Mechanism:** Independent Review Committee (IRC) for all radiographic assessments.

14. Observation & Follow-Up Schedule

Total Duration: Up to 12 cycles of treatment + Long-term follow up. **Onsite Visit Cadence:** Every 3 weeks (Q3W) during the active treatment phase. **Remote Monitoring Frequency:** Follow-up survival checks post-treatment. **Checklist Review Interval:** Regular tumor assessments (FDG-PET/CT) evaluated at defined milestones.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				